Serious vs. Non-Serious Case Assessment Report

Pharmacovigilance training report | Source: user-provided case dataset | 16 October 2026

1. Scope and Method

The attachment contains one table with multiple case records, rather than five separate files. Five distinct records were selected: SYN-PV-005, SYN-PV-008, SYN-PV-010, SYN-PV-014 and SYN-PV-019. Source seriousness labels are retained as the starting point. “Medically important” is treated as potentially serious pending review against the regulatory definition for other medically important medical events. No missing facts have been inferred.

2. Regulatory Seriousness Criteria

Criterion	Regulatory assessment	Finding in selected records
Death	The adverse event resulted in death.	No death reported.
Life-threatening	The patient was at immediate risk of death when the event occurred; theoretical potential is insufficient.	No life-threatening event reported.
Hospitalization	Initial or prolonged inpatient hospitalisation due to the event.	SYN-PV-010 and SYN-PV-014 are flagged “Hospitalisation” in the source; confirm admission details.
Persistent/significant disability	Substantial disruption of normal life functions.	No disability reported.
Other medically important event	May be serious if it jeopardises the patient or requires intervention to prevent a serious outcome.	SYN-PV-005 is marked “Medically important”; confirm clinical rationale.

3. Case Classification Matrix

Case ID	Product/reaction	Source seriousness	Assessment	Rationale/follow-up	Reporting implication
SYN-PV-005	Ciprofloxacin — tendon pain; onset day 15	Medically important	Potentially Serious*	No death, life-threatening event, hospitalization or disability stated. Review whether clinical significance meets the other-medically-important-event threshold.	If confirmed serious and reportable, apply the jurisdiction-specific expedited deadline; assess expectedness and causality.
SYN-PV-008	Losartan — fatigue; onset day 2	Not serious	Non-Serious	Source explicitly labels it not serious; no listed serious outcome is described.	Normally routine/non-expedited handling, subject to local rules and SOPs.
SYN-PV-010	Atorvastatin — myalgia; onset day 21	Hospitalisation	Serious	Hospitalization flag in source. Confirm initial/prolonged inpatient admission and obtain dates and reason.	Expedited reporting may apply when the relevant regulatory criteria are met; confirm context and expectedness.
SYN-PV-014	Sertraline — dizziness; onset day 9	Hospitalisation	Serious	Hospitalization flag in source. Confirm inpatient admission and relationship to the event.	Expedited reporting may apply when the relevant regulatory criteria are met; confirm context and expectedness.
SYN-PV-019	Metformin — nausea; onset day 9	Not serious	Non-Serious	Source explicitly labels it not serious. Dechallenge/rechallenge does not by itself determine seriousness.	Normally routine/non-expedited handling, subject to local rules and SOPs.

*Provisional: the source says “Medically important,” but the narrative does not establish the regulatory threshold. A qualified safety reviewer should document the final decision.

4. Reporting Timelines — Jurisdiction Matters

There is no single global deadline for all serious cases. The timeline depends on jurisdiction, investigational versus marketed use, expectedness, causality and the applicable reporting pathway. The following are high-level training references; verify current regulations and company SOPs.

Context	Common expedited timeline	Important qualification
US investigational drug trial: qualifying serious, unexpected suspected adverse reaction	Generally within 15 calendar days after sponsor determination; unexpected fatal or life-threatening suspected reactions as soon as possible, no later than 7 calendar days.	Not every serious adverse event automatically meets FDA IND safety-reporting criteria.
EU/EEA clinical trial SUSAR	Generally 7 days for fatal/life-threatening SUSARs, with follow-up within a further 8 days; 15 days for other SUSARs.	Confirm current EU CTR/CTIS requirements for the specific trial.
Marketed products / spontaneous reports	Varies by country, product and report type; serious suspected reactions often have expedited requirements.	Do not automatically apply clinical-trial SUSAR timelines to post-marketing cases.

Expectedness is separate from seriousness. For SYN-PV-010 and SYN-PV-014, hospitalization is only a source flag; confirm the actual inpatient admission. For SYN-PV-005, review the “medically important” designation. For the two source-labelled non-serious cases, expedited serious-case timelines would not normally apply solely on the basis of these records.

5. Follow-Up Checklist

- Hospitalization cases: admission/discharge dates, reason, and whether admission was initial or prolonged.
- Medically important event: clinical rationale, intervention required, and whether the patient was jeopardized.
- All cases: event chronology, outcome, relevant history, concomitant medicines, suspect-drug action, causality, and expectedness against the correct reference safety information.
- Confirm jurisdiction and clinical-trial versus post-marketing context before assigning a definitive deadline.

6. Conclusion

Of the five selected records, SYN-PV-010 and SYN-PV-014 are classified as serious based on the dataset’s hospitalisation flag; SYN-PV-008 and SYN-PV-019 are non-serious according to the source field; and SYN-PV-005 is provisionally potentially serious because it is marked medically important. This matrix supports preliminary training/triage only. Final regulatory decisions and deadlines require confirmation of case facts and the applicable reporting framework.

Source note: Case details were taken from the supplied dataset. Facts not stated in the source are identified as requiring follow-up.